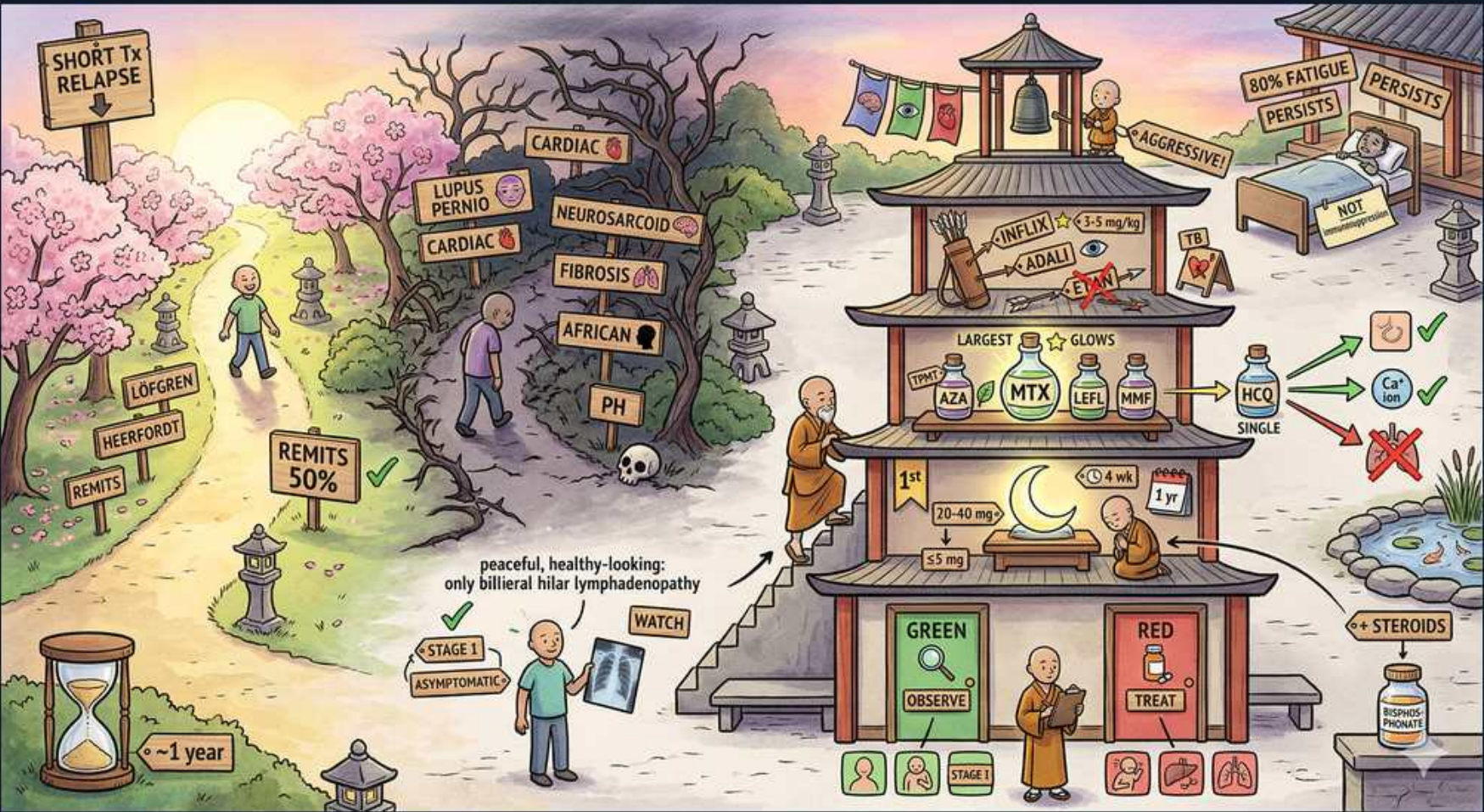


Sarcoidosis — Prognosis & Treatment



1. Löfgren remits 50% / ~1yr

WHAT YOU SEE

Hourglass and Löfgren sign, remits 50%, ~1 year

WHAT IT ENCODES

Acute sarcoid, especially Löfgren syndrome (erythema nodosum + bilateral hilar adenopathy + arthritis), has an excellent prognosis with spontaneous remission in ~80-90% within 1-2 years. First-line therapy is supportive: NSAIDs for the arthritis/fever and colchicine if needed — NOT chronic immunosuppression.

BOARD TRAP

Starting long-term steroids/methotrexate for self-limited Löfgren disease — the correct answer is NSAIDs and observation. Reaching for systemic immunosuppression in a classic Löfgren vignette is a distractor; reserve it for organ-threatening or progressive symptomatic disease.

2. Poor prognosis branches

WHAT YOU SEE

Dark tree with labels cardiac, neurosarcoid, fibrosis, lupus pernio, African, PH

WHAT IT ENCODES

Poor-prognosis features (the 'bad tree'): cardiac sarcoid, neurosarcoidosis, Stage IV pulmonary fibrosis, lupus pernio, pulmonary hypertension, chronic uveitis, and African-American race with chronic multi-organ disease. These predict persistent/progressive disease and higher mortality and warrant aggressive systemic therapy.

BOARD TRAP

Lumping all sarcoid prognosis together — the exam wants you to split GOOD prognosis (Löfgren, erythema nodosum, acute onset, Stage I) from BAD prognosis (lupus pernio, cardiac, neuro, fibrosis, PH). Mixing up erythema nodosum (good) with lupus pernio (bad) is the classic prognosis trap.

3. Lupus pernio (chronic)

WHAT YOU SEE

Lupus pernio branch on the bad tree

WHAT IT ENCODES

Lupus pernio (chronic violaceous nasal/facial plaques) is a marker of chronic, treatment-resistant, poor-prognosis disease and often responds poorly to steroids alone — anti-TNF therapy (infliximab) is frequently required.

BOARD TRAP

Treating lupus pernio with topical/short-course steroids and expecting resolution — it is refractory disease that typically needs systemic steroids plus a steroid-sparing agent or infliximab. Also recall: lupus pernio has NOTHING to do with SLE despite the name.

4. Cardiac / neuro = aggressive

WHAT YOU SEE

Cardiac and neurosarcoid branches labeled aggressive

WHAT IT ENCODES

Cardiac and neurosarcoidosis are organ- and life-threatening and demand prompt, aggressive systemic therapy — high-dose steroids plus a steroid-sparing agent (methotrexate, azathioprine) and often infliximab. Cardiac sarcoid additionally needs an ICD for arrhythmia/sudden-death risk.

BOARD TRAP

Withholding treatment from cardiac/neuro disease just because the patient looks well or the resting ECG is normal — these are the deadly forms. The right move is aggressive immunosuppression ($\pm$ ICD for cardiac), not observation. Treating cardiac sarcoid with antiarrhythmics alone, without immunosuppression and ICD evaluation, is wrong.

5. Asymptomatic Stage 1 → WATCH

WHAT YOU SEE

Calm figure with chest film, Stage 1 asymptomatic, WATCH

WHAT IT ENCODES

Asymptomatic Stage I disease (bilateral hilar lymphadenopathy ALONE, normal PFTs, no organ threat) is OBSERVED, not treated, because the spontaneous remission rate is high (~60-90%). Follow with periodic imaging and PFTs.

BOARD TRAP

Treating asymptomatic bilateral hilar adenopathy with steroids — observation is the correct answer. Giving prednisone to an asymptomatic Stage I patient exposes them to steroid toxicity for disease that usually self-resolves.

6. GREEN observe vs RED treat

WHAT YOU SEE

Green OBSERVE monk vs RED TREAT monk at pagoda base

WHAT IT ENCODES

The core treatment principle: TREAT only when disease is symptomatic, progressive, or organ-threatening (lungs with declining PFTs, eyes, heart, CNS, kidneys, hypercalcemia, disfiguring skin). Otherwise OBSERVE. Indications to treat outweigh the morbidity of long-term immunosuppression.

BOARD TRAP

Treating every sarcoid patient — many (especially asymptomatic Stage I) need only watchful waiting. Conversely, undertreating clear organ-threatening disease is the opposite error. The discriminator on exams is the presence/absence of symptoms or threatened organ function.

7. Steroids 1st line

WHAT YOU SEE

Pagoda level reading 20-40 mg, 1st line, taper

WHAT IT ENCODES

Corticosteroids are FIRST-LINE when treatment is indicated: oral prednisone ~20-40 mg/day (higher, e.g. ~1 mg/kg or IV pulse, for cardiac/neuro/ocular), continued for several months then SLOWLY tapered over 6-12+ months. Add calcium/vitamin D bone protection cautiously — but avoid vitamin D if the patient is hypercalcemic.

BOARD TRAP

Tapering steroids too quickly or treating for only a few weeks — sarcoid has a high RELAPSE rate with short courses, so treatment is months-long with a slow taper. Also a trap: routinely giving vitamin D for steroid bone protection in a patient with sarcoid hypercalcemia — that worsens the calcium.

8. MTX / AZA / steroid-sparing

WHAT YOU SEE

AZA, MTX, LEF, MMF bottles labeled largest, glows

WHAT IT ENCODES

Methotrexate is the preferred SECOND-LINE steroid-sparing agent (others: azathioprine, leflunomide, mycophenolate). Used when patients cannot taper off steroids, have steroid side effects, or have refractory disease. Methotrexate requires folic acid supplementation and monitoring for hepatotoxicity, cytopenias, and pneumonitis.

BOARD TRAP

Using methotrexate as the FIRST-line agent — steroids come first; methotrexate is the steroid-sparing add-on. Also remember MTX is contraindicated in pregnancy (teratogen) and in significant liver disease — azathioprine is the alternative in those settings.

9. Anti-TNF (infliximab/adalimumab)

WHAT YOU SEE

INFLIX/ADALI 3-5 mg/kg label at top pagoda

WHAT IT ENCODES

TNF-alpha inhibitors — infliximab (~3-5 mg/kg) and adalimumab — are reserved for REFRACTORY disease and severe phenotypes: lupus pernio, neurosarcoidosis, and refractory cardiac/ocular disease. They work because TNF-alpha is central to maintaining the granuloma.

BOARD TRAP

Starting anti-TNF therapy without first SCREENING for and excluding latent TB (and hepatitis B) — TNF inhibitors reactivate latent TB, which is especially dangerous given that TB is the main granulomatous mimic of sarcoid. Check a PPD/IGRA before infliximab; remember sarcoid causes anergy so a negative skin test can be falsely reassuring.

10. Hydroxychloroquine

WHAT YOU SEE

HCQ single label on the right

WHAT IT ENCODES

Hydroxychloroquine is useful for CUTANEOUS sarcoidosis and for sarcoid-related HYPERCALCEMIA (it reduces 1,25-vitamin D production). It is a steroid-sparing option for skin and mucosal disease and neurosarcoid adjunct.

BOARD TRAP

Starting hydroxychloroquine without a baseline and periodic ophthalmologic exam — it carries a risk of irreversible retinal toxicity (bull's-eye maculopathy). Screening eye exams are mandatory, which is doubly relevant in sarcoid where uveitis already threatens vision.

11. Calcium / monitoring

WHAT YOU SEE

Right-side icons for Ca and labs with checkmarks

WHAT IT ENCODES

Monitor serum/urine calcium, renal and liver function, PFTs/DLCO, and EKG over time. Sarcoid hypercalcemia (from granuloma 1-alpha-hydroxylase making active vitamin D, with a SUPPRESSED PTH) is managed with corticosteroids, hydroxychloroquine, hydration, and AVOIDING vitamin D, calcium supplements, and excess sun.

BOARD TRAP

Giving vitamin D / calcium supplements or recommending sun exposure to a hypercalcemic sarcoid patient — this worsens hypercalcemia. Also do not work it up as primary hyperparathyroidism: in sarcoid the PTH is LOW (PTH-independent calcitriol excess), not high.

12. Short tx → relapse

WHAT YOU SEE

Sign reading SHORT TX RELAPSE on the left path

WHAT IT ENCODES

Inadequate treatment duration or overly rapid steroid tapering leads to RELAPSE. Effective therapy generally lasts many months (often 6-12+ months) with a slow taper; relapse is common and may require resuming or escalating therapy.

BOARD TRAP

Stopping steroids after a few weeks once symptoms improve — this leads to a high relapse rate. The exam answer is a prolonged course with gradual taper, not a short burst.

13. Fatigue persists

WHAT YOU SEE

Bedside figure labeled fatigue persists

WHAT IT ENCODES

Fatigue is a major, often persistent symptom that can continue despite well-controlled organ inflammation. Contributors include systemic inflammation, small-fiber neuropathy, depression, sleep disturbance, and corticosteroid side effects.

BOARD TRAP

Escalating immunosuppression just for residual fatigue — it frequently does not correlate with active granulomatous disease, so more steroids/biologics add toxicity without benefit. Evaluate for treatable contributors (small-fiber neuropathy, hypothyroidism, sleep apnea, depression) instead.

14. + Steroids endpoint

WHAT YOU SEE

Sign reading + steroids on the right

WHAT IT ENCODES

Add systemic corticosteroids whenever organ-threatening disease is identified — cardiac, neuro, ocular (sight-threatening), renal, severe pulmonary with declining function, or symptomatic hypercalcemia. Severe forms warrant higher doses (~1 mg/kg or IV pulse) plus a steroid-sparing agent.

BOARD TRAP

Reflexively adding steroids for any abnormality (e.g., asymptomatic mild PFT change or stable Stage I adenopathy) — steroids are for symptomatic/organ-threatening disease, not for every imaging or lab finding. Match the intensity of therapy to organ threat.